

Neutral Citation: 2016 ONFSCDRS 98

FSCO A13-002459
and A13-002460

BETWEEN:

RUQIA RAHIM ALI and BAKIDAR FEROZUDDIN

Applicants

and

CERTAS DIRECT INSURANCE COMPANY

Insurer

REASONS FOR DECISION

Before: Alec Fadel

Heard: June 1, 3, 4, 2015 at the Financial Services Commission, written submissions received up to and including March 17, 2016

Appearances: Mr. Shahan Alexanian for Ms. Ali and Mr. Ferozuddin
Jean-Claude Rioux for Certas Direct Insurance Company

Issues:

The applicants, Bakidar Ferozuddin and Ruqia Rahim Ali, husband and wife, were injured in a motor vehicle accident on October 5, 2011 (the accident). They applied for and received statutory accident benefits from Certas Direct Insurance Company (“Certas”), payable under the *Schedule*.¹ Certas terminated Mr. Ferozuddin’s weekly income replacement benefits effective March 26, 2012, and denied a number of treatment plans relating to medical benefits for each

¹The Statutory Accident Benefits Schedule — Effective September 1, 2010, Ontario Regulation 34/10, as amended.

applicant. The parties were unable to resolve their disputes through mediation, and Mr. Ferozuddin and Ms. Ali applied for arbitration at the Financial Services Commission of Ontario under the *Insurance Act*, R.S.O. 1990, c.I.8, as amended.

The issues in this hearing are:

With regard to Mr. Ferozuddin:

1. Is Mr. Ferozuddin entitled to an income replacement benefit from March 26, 2012 to June 11, 2012 at a rate of \$400.00 per week?
2. Do Mr. Ferozuddin's impairments fall under the Minor Injury Guideline?
3. Is Mr. Ferozuddin entitled to the following medical benefits:
 - i) \$2,521.13 for chiropractic treatment as set out on a treatment plan dated October 25, 2011
 - ii) \$990.00 for massage therapy as set out on a treatment plan dated October 25, 2011
 - iii) \$650.00 for massage therapy on a treatment plan of November 28, 2012
 - iv) \$1,462.50 for chiropractic treatment on a treatment plan dated November 28, 2012?
4. Is Mr. Ferozuddin entitled to interest on any overdue benefits?
5. Is either party entitled to their expenses of the arbitration?

With regard to Ms. Ali:

1. Do Ms. Ali's impairments fall under the Minor Injury Guideline?
2. Is Ms. Ali entitled to the following medical benefits:
 - i) \$501.93 for massage therapy as set out on a treatment plan dated December 11, 2012
 - ii) \$1,300.00 for chiropractic treatment as set out on a treatment plan dated December 11, 2012
 - iii) \$2,521.13 for chiropractic treatment on a treatment plan dated October 28, 2011
 - iv) \$990.90 for chiropractic treatment on a treatment plan dated October 28, 2011?

3. Is Ms. Ali entitled to interest on any overdue benefits?
4. Is either party entitled to their expenses of the arbitration?

Result:

With regard to Mr. Ferozuddin:

1. Mr. Ferozuddin is entitled to an income replacement benefit from March 26, 2012 to June 11, 2012 at a rate of \$400.00 per week, plus applicable interest.
2. Mr. Ferozuddin's impairments do not fall under the Minor Injury Guideline.
3. Mr. Ferozuddin is not entitled to the medical benefits in dispute.
4. Mr. Ferozuddin is entitled to his expenses of the arbitration.

With regard to Ms. Ali:

1. Ms. Ali's impairments do not fall under the Minor Injury Guideline.
2. Ms. Ali is not entitled to the medical benefits in dispute.
3. There is no interest that is payable.
4. Ms. Ali is entitled to her expenses of the arbitration.

During the course of the hearing, the insurer requested that I not rely on the clinical notes and records of Dr. Mah, family doctor, since he was not called as a witness by the applicants and it did not have an opportunity to conduct a cross-examination. Dr. Mah's notes appeared in the applicants' brief, delivered to the insurer at least 30 days before the hearing. There was no indication from the applicants that they intended to call Dr. Mah as a witness (in the pre-hearing letter or otherwise). The insurer has not shown any evidence that it informed the applicants that it intended to cross-examine Dr. Mah, which would have given the applicants notice to summons this witness. The insurer's motion is therefore denied.

The applicants objected to the admission of Dr. Platnick's report dated March 29, 2012 because this report, although in the insurer's document brief, was not properly indexed and could not be identified as a document that it intended to rely upon. I admitted this report over their objection.

EVIDENCE AND ANALYSIS:

Background:

Mr. Ferozuddin was operating a motor vehicle with Ms. Ali in the front passenger seat when the vehicle was T-boned on the front passenger side by a vehicle exiting a driveway. Ms. Ali felt dazed after the accident but there was no loss of consciousness. Her chest impacted the dashboard and her right knee and hand struck something in the vehicle. She remained in the vehicle until the ambulance attendants assisted her out of the vehicle. Ms. Ali and her children, who were also in the vehicle, were taken by ambulance to North York General Hospital. Mr. Ferozuddin was able to exit the vehicle independently. He did not go with his family in the ambulance and instead was picked up by a friend who took him to the hospital where his family was.

Ms. Ali sustained injury to her neck and trapezius area where she had pain. She also had pain in her back (lower and upper), right knee and hand pain. She had x-rays taken at the hospital that showed no fractures. She was not given prescription medication at the time and it is noted that she was breastfeeding. Mr. Ferozuddin did not feel particular focal pain after the accident but over the next few days became stiff with pain in the low and upper back, neck and shoulders (trapezius). The applicants attended at their family doctor and began a course of treatment.

The insurer funded therapy and took the position that the injuries sustained in the accident for each applicant were predominantly minor as per the Minor Injury Guideline (MIG) and therefore their entitlement to medical benefits was capped at \$3,500 each. The applicants contend that they needed further treatment because of pain that continued beyond the normal recovery time for soft-tissue injuries. They point to their chronic pain diagnosis as proof that their injuries were not

predominantly minor and therefore that the MIG does not apply. The insurer argues that the development of certain types of chronic pain may take one out of the MIG, but this is not the situation in this case.

For the reasons set out below, I find that both Mr. Ferozuddin and Ms. Ali sustained chronic pain as a result of the accident. I find that their chronic pain as described by Dr. Blitzler, is not a condition to be treated under the MIG and that the applicants' injuries are not predominantly minor. However, based on the evidence, I find that the applicants are not entitled to the medical benefits at issue in this arbitration because they have failed to show that they are reasonable and necessary as a result of injuries sustained in the accident. Mr. Ferozuddin is entitled to his claim for income replacement benefits as he sustained a substantial inability to complete his pre-accident employment as a truck driver.

Do the applicants suffer from chronic pain as a result of the motor vehicle accident?

Mr. Ferozuddin:

Mr. Ferozuddin attended at his family doctor, Dr. Mah, several times after the accident complaining of pain. On October 20, 2011, the clinical notes describe the accident and note bilateral trapezius, mid/low back tenderness with recommendations to take ibuprofen. The March 27, 2012 note refers to the IRB termination and states "still pain both neck and mid/low back sore, not working . . . tender bilateral trapezius, upper back and low back tenderness." He was given sample packets of Vimovo to take for pain. There are several appointments where accident-related injuries are not mentioned but again in November 2014, Dr. Mah refers to upper and lower back pain and occasional headaches. It also noted that he was still attending therapy once a month relating to the accident. The notes again refer to trapezius bilateral, stiffness lower lumbar with mild pain on extension and the assessment is noted as "chronic neck and back pain." Cymbalta samples were given at this appointment. Again in March 2015, the applicant attended complaining of neck and low back pain/stiffness. While Dr. Mah did not testify to explain his impressions and speak to his notes, I take them to show that Mr. Ferozuddin

attended to his family doctor several times since the accident complaining of pain from injuries that seem to be related to the accident.

Dr. Blitzer examined Mr. Ferozuddin on April 21, 2015 (more than 3 years after the accident) and reviewed the clinical notes and records of Finch Physiocare Clinic and Dr. Mah. He concluded that Mr. Ferozuddin had musculoskeletal impairments and chronic pain, the latter condition due to the persistence of symptoms and the passage of time without improvement. Dr. Blitzer diagnosed:

1. Mechanical and myofascial lumbosacral back pain with associated sacroiliac joint irritation
2. Thoracic back spinal and myofascial pain
3. Cervical strain
4. Trapezius myofascial pain
5. Left lower extremity symptoms may be multifactorial with potential radicular, referred, and also local injury/strain factors
6. Headaches post-accident
7. Sleep disruption by pain
8. Fatigue/reduced energy, not assessed in detail, is often related to the draining effect of persistent pain, non-restorative sleep
9. Chronic pain. Mr. Ferozuddin has persistent pains for over 3-1/2 years now and associated problems
10. Loss of function with regards to Activities of Daily Living and alteration in life situation due to problems and diagnoses above.

Dr. Blitzer was of the view that Mr. Ferozuddin's current problems were directly related to the accident. He found Mr. Ferozuddin significantly better (from earlier injuries) and also found that the onset of problems was temporally related to the accident. Dr. Blitzer also noted that Mr. Ferozuddin was a credible historian showing good effort during the clinical examination with no pain magnification.

Dr. Blitzer opined that Mr. Ferozuddin had chronic pain and that chronic pain was a different diagnosis from the original musculoskeletal diagnoses because the latter can be short-term but chronic pain means that there is another process going on that is perpetuating the pain.

While Dr. Platnick assessed Mr. Ferozuddin on at least 2 occasions on behalf of the insurer², he did not comment on chronic pain in either of these two assessment reports. Given that this is the question before me, I do not find his reports of much assistance in my analysis concerning chronic pain and I give his reports little weight in this regard.

Ms. Ali:

Ms. Ali attended at her family doctor, Dr. Mah, to treat her injuries from the accident. The clinical notes of October 20, 2011 detail the accident and note that she was complaining of neck/shoulders/back and chest pain, she had a bruised right knee and headaches. Dr. Mah noted bilateral trapezius tenderness/spasm, mid and lower back spasms, and residual bruising. She was advised to take Advil. She attended again on December 15, 2011 and the notes show that she still had pain in her neck, shoulders, mid- and low-back and was attending therapy once per week. The notes of her attendance on March 27, 2012, show she was still attending therapy and was still sore at the base of her neck, left trapezius, left rhomboid and lower back tenderness, and that the lower back was better. Ms. Ali was given samples of Vimovo and indicated to continue therapy. Further notes from December 4, 2012, October 20, 2012, November 7, 2014 and March 27, 2015 all reference issues of pain concerning injuries from her accident.

Dr. Blitzer assessed Ms. Ali on April 21, 2015 and confirmed her initial primary injuries after the accident including headaches.

Dr. Blitzer also confirmed that Ms. Ali's worst problems at the time were pain in the shoulder, neck and back noting her complaint of constant left trapezius pain with lesser pain in the right

²March 6, 2012 in relation to his claim for an IRB, and a paper review completed on March 29, 2012 in relation to an OCF-18 dated February 23, 2012 in the amount of \$1,462.50 for 9 sessions of physical rehabilitation and 9 sessions of massage therapy.

trapezius area, daily pain in the neck posteriorly bilaterally and daily low back pain centered in the lower lumbosacral area, which spread up and down to both sides. This pain increased with activity. She also had daily mid-back pain.

Dr. Blitzer also noted that Ms. Ali had headaches since the accident about three times a week that lasted a few hours. Ms. Ali indicated that she took Tylenol extra strength or Tylenol muscle formula around three times a week. He noted that she had poor sleep with intermittent waking. Ms. Ali estimated that she was about 20% improved since the accident. Dr. Blitzer noted that Ms. Ali had not experienced any improvement in the year prior to his assessment and felt she has reached a plateau.

Dr. Blitzer confirmed the diagnoses for all of Ms. Ali's complaints and also diagnosed chronic pain because of her persistent pain in multiple sites for 3-½ years with associated problems.

Dr. Blitzer noted that Ms. Ali was a credible historian providing good effort for the clinical examination with no pain magnification behaviours. He concluded that her current problems were compatible with the mechanism of trauma she sustained in the accident.

In his testimony, Dr. Windsor, who assessed Ms. Ali in relation to a non-earner benefit on behalf of the insurer in early 2012 agreed that he had not addressed the issue of whether Ms. Ali suffered from chronic pain. He also testified that he is aware that a certain percentage of accident victims who sustain soft tissue injury could go on to develop chronic pain.

Conclusion:

On balance, the applicants have provided credible medical evidence to support that they have developed chronic pain as a result of the accident. The insurer has not responded to these medical opinions with its own expert on the issue of chronic pain. Dr. Blitzer, a physician for the past 30 years with a focus on complex cases and chronic pain for the last fifteen, opined that the applicants developed chronic pain. In addition, the applicants both described their pain complaints in a straightforward convincing manner and did not contradict themselves. Also, the

clinical notes and records of Dr. Mah support that the applicants attended since the accident and until at least March 2015 complaining of accident-related pain issues. Given the totality of this evidence, I find that the applicants have developed chronic pain as a result of the accident.

Is chronic pain meant to be treated under the Minor Injury Guideline?

The MIG places a limit on available benefits if an injury can be classified as minor. Subsections 3(1) and 18(1) of the *Schedule* state that any injury that is predominantly a “sprain, strain, whiplash associated disorder, contusion, abrasion, laceration or subluxation and any clinically associated sequelae” is eligible to a maximum of \$3,500 in medical and rehabilitation benefits.

The MIG is specific when it states in the Introduction that:

The SABS and this Guideline are intended to encourage and promote the broadest use of this Guideline, recognizing that most persons injured in car accidents in Ontario sustain minor injuries for which the goods and services provided under this Guideline are appropriate.

Part 3 of the MIG states that, “an insured person’s impairment comes within this Guideline if the impairment is predominantly a minor injury.”

Therefore, the applicants must prove that their injuries were predominantly not minor. In this case, that means showing that their chronic pain condition exists, and is not a condition that was contemplated as being included in the MIG as “clinically associated sequelae”.

Dr. Blitzer distinguished a chronic pain condition from the soft tissue injuries that an individual may suffer after a motor vehicle accident. He characterized chronic pain as a complication that arises when the soft tissue injuries do not heal as expected in the weeks or months following the injury, hence “a new condition.” Specifically, he described chronic pain as an “aberrant physiology” in the form of a negative feedback loop between a patient’s nerves and their brain, resulting in a hypersensitivity to pain beyond what would normally be expected as a result of the injury.

In his testimony, Dr. Blitzer explained that most soft tissue injuries heal and that only a small proportion of injuries do not heal properly and develop a chronic pain physiology. Soft tissue injuries refer to strains, sprains, bruising and short-term inflammation of the soft-tissue structures being muscles ligaments, tendons, and connective tissue. A soft-tissue injury, according to Dr. Blitzer, would have a short-to-medium duration healing in weeks or a few months. He stated that some soft-tissue injuries do not fully heal and lead to chronic pain which is a “different situation” from the actual original injury. He testified that this happens in a minority of cases and is not the “normal expected outcome.” Dr. Blitzer stated that this was aberrant. Instead of the body’s pain receptors turning off, as is expected, they are turned on more, “a physiology gone wrong.”

Dr. Blitzer testified that normally there is some benefit to pain in the initial phases of an injury as it tells the body it has been injured, protecting the area. However, with chronic pain the pain signal does not go away even though the initial shearing and tearing in the muscle will start to heal, the pain will actually escalate. Instead of the body perceiving there is some pain and in the normal circumstance the signal stops being sent, the body actually starts making more of these neurotransmitters in the area to send more pain signals. In addition, the body actually physiologically makes more nerve ending receptors to pick up the pain signals. This actually generates more pain in the area even though the original injury has subsided. Because the “feedback loop” gets disrupted and the pain signal fails to stop, there is peripheral sensitization or an increased sensitivity to pain in the peripheral tissues (the local area that was originally injured). As a result, a stimulus that would normally not be expected to be painful, like lighter touch or movement, will induce pain because there is hypersensitivity in the area.

When looking at the enumerated list in the MIG for injuries that would put an applicant into the MIG, they all have a characteristic of an immediate injury. The MIG is meant to treat injuries that respond to fast treatment of short duration. These two objectives are clearly set out in the MIG’s introduction:

- a) Speed access to rehabilitation for persons who sustain minor injuries in auto accidents;

- d) Be more inclusive in providing immediate access to treatment without insurer approval for those persons with minor injuries as defined in the SABS and set out in Part 2 of this Guideline.

When looking at the blocks of treatment, most treatment for minor injuries is envisioned to take place in the 12 weeks following the accident. The evidence before me is that chronic pain is not identifiable in the patient until approximately 6 months after the accident and as late as one year after the accident. It is therefore hard to see how the MIG was meant to deal with the treatment of chronic pain when it cannot even be identified until after the MIG limits will have been exhausted (in many cases). It therefore seems incorrect to suggest that the MIG was meant to deal with chronic pain as “any clinically associated sequelae”.

Further, long before the MIG was drafted, chronic pain has been recognized by the Courts. In the Supreme Court of Canada decision *Nova Scotia (Workers’ Compensation Board) v. Martin*,³ Justice Gontier, in his introduction, acknowledged that there was no authoritative definition of chronic pain, the Court went on to say:

It is, however, generally considered to be pain that persists beyond the normal healing time for the underlying injury or is disproportionate to such injury, and whose existence is not supported by objective findings at the site of the injury under current medical techniques. Despite this lack of objective findings, there is no doubt that chronic pain patients are suffering and in distress, and that the disability they experience is real. While there is at this time no clear explanation for chronic pain, recent work on the nervous system suggests that it may result from pathological changes in the nervous mechanisms that result in pain continuing and non-painful stimuli being perceived as painful. These changes, it is believed, may be precipitated by peripheral events, such as an accident, but may persist well beyond the normal recovery time for the precipitating event. Despite this reality, since chronic pain sufferers are impaired by a condition that cannot be supported by objective findings, they have been subjected to persistent suspicions of malingering on the part of employers, compensation officials and even physicians.

The Supreme Court of Canada found that the regulation in question clearly imposed differential treatment upon injured workers suffering from chronic pain on the basis of the nature of their

³[2003] 2 SCR 504

physical disability, an enumerated ground under s. 15(1) of the *Charter*. While my decision considers a different regulation, I refer to this case for the fact that chronic pain has been recognized by the highest court as a condition that is real and that may persist beyond the normal recovery time.

In light of all of this information, I find it unlikely that the MIG is intended to include treatment for chronic pain by reference to “any clinically associated sequelae.” It seems more likely that if chronic pain was intended to be dealt with under the MIG, it would be listed as a specific condition or injury in the *Schedule*.

Finally, the MIG is emphatic that most individuals in auto accidents in Ontario sustain minor injuries and that the *Schedule* and the Guideline are intended to promote the broadest use of the Guideline. In fact, Dr. Blitzer testified that chronic pain is not the normal expected outcome and is in fact aberrant. He stated that the condition will appear in only a minority of cases and was essentially a physiology gone wrong.

I find that the applicants have chronic pain as a result of the accident and that the injuries sustained were not primarily minor. Chronic pain was diagnosed by Dr. Blitzer who described it as a condition that arises because of an aberrant physiology. This condition is not listed in s. 3(1) and 18(1) of the *Schedule* as a minor injury and the evidence supports that it is not a clinically associated sequelae. Therefore, the applicants’ injuries do not fall under the MIG.

Are the applicants entitled to the medical benefits claimed?

Mr. Ferozuddin attended therapy at Finch Physiocare Clinic after the accident. He received physical rehabilitation treatments, massage therapy and acupuncture. This treatment continued until May 2012, subsequent treatment plans were denied by the insurer, presumably because he had reached the MIG limits of \$3,500.00. Mr. Ferozuddin reported receiving pain relief from the treatment he received. The treatment was focused on his lower back, upper back and shoulders. He testified that he would have continued treatment but was unable to afford to do so when the insurer stopped funding.

Ms. Ali received massage treatment and other modalities at Finch Physiocare Clinic. In her testimony, she indicated that she felt relief from pain and was able to better take care of her children when she returned home after the treatment. She specifically indicated that the TENS machine would help soften her muscles. She testified that she stopped her therapy when the insurer refused to pay because she could not afford to pay out of pocket. Ms. Ali testified that her condition has worsened since the accident.

It is not automatic that when an applicant is out of the MIG they are entitled to all medical and rehabilitation benefits beyond \$3,500.00. The test set out in the *Schedule* at s. 15 is that the medical benefits be “reasonable and necessary.” The onus is always on the applicant to prove entitlement to the specific benefits that they are claiming. The only evidence I have supporting massage and chiropractic treatment is the applicants’ testimony that they were experiencing pain relief from the treatment. Dr. Mah’s clinical notes document the pain complaints and that the applicants were attending treatment but do not specifically state that he was recommending the treatment and he did not testify to clarify.

Dr. Blitzer makes recommendations in his report for physical rehabilitation for each applicant without details as to the modality. However, I do not find his evidence supportive of the massage and chiropractic treatment in dispute before me. In his report, Dr. Blitzer notes for each applicant that the treatments they had received to date were appropriate. I find that the evidence of Dr. Blitzer does not support that the treatment in dispute is reasonable and necessary.

Moreover, I am puzzled that there are two treatment plans for each applicant that are dated the same month of the accident. It was never explained to me why these claims were being made when presumably coverage for treatment under the MIG was available.

The applicants did not file the treatment plans at the hearing, though I invited the parties to make submissions on potentially allowing this evidence to be admitted late. My concern was that I was asked to award treatment from treatment plans that I was unable to review. The treatment plan may have included costs for assessments or other goods and services which were not clarified at

the hearing. However, after reviewing the evidence and submissions on that issue, I find that the applicants have failed to prove entitlement to the medical benefits claimed, in any event, and my receiving the disputed treatment plans would not affect my disposition on this issue.

I find there is insufficient evidence to support an order for the medical benefits in dispute.

Is Mr. Ferozuddin entitled to an income replacement benefit as claimed?

At the time of the accident, Mr. Ferozuddin was employed as a truck driver. His duties involved delivering lumber across the GTA, to a depot located in Scarborough. His job duties included driving the truck between locations, loading and unloading, by hand, lumber that weighed between 20 to 22 pounds, and operating a forklift for larger loads.

Approximately 11 weeks after the termination of his IRB, Mr. Ferozuddin began employment as a forklift operator at Don Foods on June 1, 2012. He testified that he had to return to employment due to the termination of his income replacement benefits. Mr. Ferozuddin testified that he was unable to return to his pre-accident employment at Timber Mart because he was unable to handle driving for more than 10 to 15 minutes because of his back pain and could not complete the physical work. He indicated that the job at Don Foods did not require driving, although it did require lifting loads between 20 to 25 pounds. Mr. Ferozuddin indicated that he struggled with the lifting aspect of the job at Don Foods and eventually stopped working there as a result. Subsequent to his job at Don Foods, Mr. Ferozuddin began working at Loblaws where he also found it difficult to handle the physical demands of that job. He ultimately took a job at Nike Canada where he was working at the time of the hearing.

Mr. Ferozuddin's evidence is that his return to work since the accident has been very difficult as a result of his accident-related injuries. He testified that he returned to work in June 2012 out of necessity since the insurer stopped his IRB. He could not, however, return to his pre-accident employment mostly because of the driving requirements which he could not complete because of his back pain. He also referred to the heavy lifting aspect of his pre-accident employment but

agreed that this requirement was present in the subsequent employment and he was able to do it, with difficulty.

Dr. Blitzer testified that sitting in a fixed position and the vibrations of the truck would exacerbate Mr. Feruzuddin's symptoms of pain. He also indicated that the torque involved in tightening down the loads could cause flare-ups and that Mr. Ferozuddin would have difficulties performing these tasks. This information was also confirmed in his report.

The notes of Dr. Mah around the time of termination of the IRB support that Mr. Ferozuddin continued to complain of accident related injuries.

Dr. Platnick, in his report of March 6, 2012 addressed Mr. Ferozuddin's entitlement to an income replacement benefit. Dr. Platnick noted in his report that Mr. Ferozuddin reported being able to drive only short distances and felt nervous doing so. Despite this, Dr. Platnick stated that Mr. Ferozuddin had an essentially normal physical examination and he did not identify any accident related impairment that would cause him to suffer a substantial inability to perform the essential tasks of a full-time short haul truck driver. Dr. Platnick did not reconcile the complaints documented by Mr. Ferozuddin at the assessment with his conclusions that there were no valid indicators to support ongoing accident-related injury or impairment. In addition, he was not available to testify as he was not called as a witness. Further, in light of my finding that Mr. Ferozuddin in fact went on to develop chronic pain, I assign little weight to Dr. Platnick's report.

I find that the evidence supports that Mr. Ferozuddin had a substantial inability to complete the essential tasks of his pre-accident employment as a long-haul driver because he could not complete the driving element of that employment. The fact that he attempted other more physical jobs is irrelevant because there was not the same driving requirement in the new employment. I find that Mr. Ferozuddin is entitled to the claimed income replacement benefit.

EXPENSES:

The applicants were successful in proving that their injuries did not fall under the MIG and that they suffer from chronic pain. This issue was novel. Although the applicants were not successful in proving entitlement to the medical benefits claimed, I find that they have had a greater degree of success than the insurer. I find that the applicants are entitled to their reasonable expenses of the arbitration. If the parties are unable to agree on a quantum of expense, they may apply for a hearing on that issue as per the *DRPC*.

Alec Fadel
Arbitrator

March 23, 2016
Date



Neutral Citation: 2016 ONFSCDRS 98

FSCO A13-002459
and A13-002460

BETWEEN:

RUQIA RAHIM ALI and BAKIDAR FEROZUDDIN

Applicants

and

CERTAS DIRECT INSURANCE COMPANY

Insurer

ARBITRATION ORDER

Under section 282 of the *Insurance Act*, R.S.O. 1990, c.I.8, as amended, it is ordered that:

1. Mr. Ferozuddin is entitled to an income replacement benefit from March 26, 2012 to June 11, 2012 at a rate of \$400.00 per week, plus applicable interest.
2. Mr. Ferozuddin injuries do not fall under the Minor Injury Guideline.
3. Mr. Ferozuddin is entitled to his expenses of the arbitration.
4. Ms. Ali's injuries do not fall under the Minor Injury Guideline.
5. Ms. Ali is entitled to her expenses of the arbitration.

March 23, 2016

Alec Fadel
Arbitrator

Date